Brigatinib monotherapy · ALK-positive advanced non-small-cell lung cancer (NSCLC) Brigatinib on its own for adults with ALK-positive advanced non-small-cell lung cancer: either after crizotinib (00562a) or before any ALK inhibitor (00562b). This is not an adjuvant course or a general pathway after every ALK inhibitor.

NATIONAL CANCER CONTROL PROGRAMME (NCCP) 00562 · VERSION 4

Brigatinib Therapy

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Columns show sequence, not equal durations. Continuous tablets do not create a chemotherapy cycle. The team confirms booking, review and any extra tests.

Check	Before starting	First seven days	Days 8–15 and first three months	Continuing treatment	Pause / stop / change
Breathing	Pulmonary function and previous lung disease review	Report new cough or breathlessness immediately	Planned clinical checks on days 8 and 15	Monthly clinical assessment or sooner if symptoms	Follow recovery; early pneumonitis can prohibit escalation
Liver tests	Baseline liver profile	Early extra tests if clinically needed	Every two weeks for first three months	As clinically indicated afterwards	Review abnormal tests before any restart
Heart and blood pressure	Baseline electrocardiogram (ECG) and blood pressure	Symptoms require earlier assessment	Every two weeks during initial treatment period	Monthly or as clinically indicated	Review heart-rate/blood pressure (BP) medicines after treatment changes
Blood, kidneys and metabolic tests	full blood count (FBC), renal profile, fasting glucose, creatine phosphokinase (CPK), amylase/ lipase	Extra tests if symptoms occur	Monthly planned tests	Continue monthly or sooner if indicated	Resolve abnormalities and review dose limitations
Benefit, medicines and priorities	ALK test, previous treatment, interactions and consent	Confirm tablet supply and contacts	Check adherence, interactions, symptoms and daily function	Disease assessment and scans per Consultant plan	Agree next treatment, follow-up and ongoing precautions

THE WHOLE TREATMENT JOURNEY

Brigatinib Therapy: treatment and checks

ALK-positive advanced NSCLC · before an ALK inhibitor or after crizotinib

The usual starting plan is 90 mg once daily for seven days, then 180 mg once daily if your team confirms the increase. Some people need a different dose. Treatment continues while helping and tolerable, with regular reviews and no fixed number of cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First seven days	Days 8–15 and first three months	Continuing treatment	Pause / stop / change
Tablets and lead-in NCCP 00562	Confirm indication and written dose	Usual 90 mg daily lead-in	Increase only if authorised; usual ongoing dose 180 mg daily	Daily treatment while beneficial and tolerated	Reason-specific restart; do not self-escalate
Breathing NCCP 00562 · SMPC	Pulmonary function and previous lung disease review	Report new cough or breathlessness immediately	Planned clinical checks on days 8 and 15	Monthly clinical assessment or sooner if symptoms	Follow recovery; early pneumonitis can prohibit escalation
Liver tests NCCP 00562	Baseline liver profile	Early extra tests if clinically needed	Every two weeks for first three months	As clinically indicated afterwards	Review abnormal tests before any restart
Heart and blood pressure NCCP 00562	Baseline ECG and blood pressure	Symptoms require earlier assessment	Every two weeks during initial treatment period	Monthly or as clinically indicated	Review heart-rate/BP medicines after treatment changes
Blood, kidneys and metabolic tests NCCP 00562	FBC, renal profile, fasting glucose, CPK, amylase/lipase	Extra tests if symptoms occur	Monthly planned tests	Continue monthly or sooner if indicated	Resolve abnormalities and review dose limitations
Benefit, medicines and priorities NCCP 00562 · SMPC	ALK test, previous treatment, interactions and consent	Confirm tablet supply and contacts	Check adherence, interactions, symptoms and daily function	Disease assessment and scans per Consultant plan	Agree next treatment, follow-up and ongoing precautions

What each check is for

1 Check the exact indication

Before the first dose

Sources: NCCP 00562

2 Early pulmonary surveillance

First week and scheduled day 8 / day 15 review

Sources: NCCP 00562 · SMPC

3 Ongoing tests

Every two weeks initially for selected tests; monthly for others

Sources: NCCP 00562

4 Restart or stop review

Any pause, dose change or final dose

Sources: NCCP 00562 · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29

Source reconciled; independent clinical, pharmacy and patient/PPI review pending